

WHO guideline on HIV service delivery

Updated guidance on the integration of diabetes,
hypertension and mental health services, and interventions
to support adherence to antiretroviral therapy

Web Annex C.

Client preferences survey



World Health
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Client preferences on integration of HIV, noncommunicable diseases and mental health services and interventions to support adherence: results of a global survey to inform WHO guidelines

Background

People living with HIV often face intersecting challenges involving noncommunicable diseases (NCDs), mental health conditions and sustained treatment adherence. To inform the World Health Organization (WHO) guidelines on HIV service delivery, a global community-led survey was conducted by WHO and Global Network of People Living with HIV (GNP+) to capture client preferences on service integration and interventions to support adherence.

Methods

A mixed-methods online survey was disseminated globally in March–April 2025. A total of 264 people living with HIV submitted complete responses. The survey included structured questions on service preferences and open-ended items about care experiences. Quantitative data were analysed descriptively; qualitative responses were thematically coded.

Results

Respondents emphasized the importance of respectful, nonjudgemental care and voiced strong concerns about stigma in referral-based models. Most preferred “one-stop” models for screening and managing NCDs (58% agreed/strongly agreed), and mental health conditions (57% agreed/strongly agreed) at their HIV clinic with regular screening offered during routine ART visits coupled with ongoing care. Integrated services were also seen as helping respondents to re-engage with care and treatment (64% of respondents preferred integrated care among other options). There was no clear preference on whether services should be community- or clinic-based. Respondents noted detailed counselling by health provider and support from peers in their community as important components of receiving NCD and mental health care. Regarding mental health services, while 43% of respondents agreed and strongly agreed that stigma was not an important consideration for offering integrated mental health care, 44% responded that integration could reduce stigma and discrimination.

Regarding interventions to support ART adherence, 98 respondents provided their experiences. The most frequently reported barriers were forgetfulness (48%), depression (33%), treatment fatigue (32%) and clinic distance (26%). A total of 122 respondents ranked the most helpful interventions to support adherence as follows: counselling and treatment literacy/education (39% and 41% respectively), followed by peer programmes (26%) and reminder tools (31%). Just over a third of the 117 respondents strongly agreed that health care providers should help address both medical and nonmedical barriers to adherence; 30% emphasized being involved in selecting adherence support options.

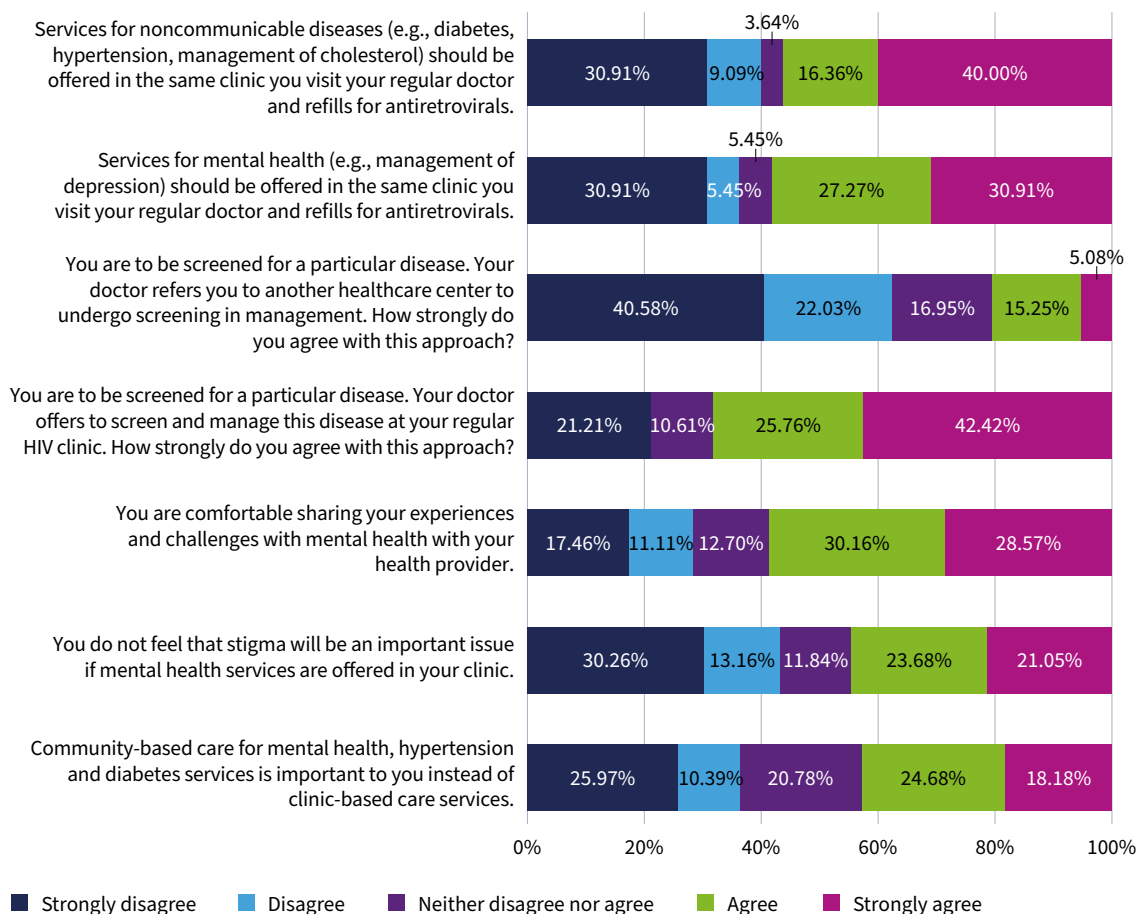
Conclusion

People living with HIV strongly support integration of HIV, NCD and mental health services – delivered in client-centred, stigma-free settings – as well as tailoring the provision of adherence support interventions to people’s needs and reported barriers. These insights are essential when shaping the WHO guidelines on HIV service delivery to ensure that care meets the real-world needs of the people most affected.

Graphs

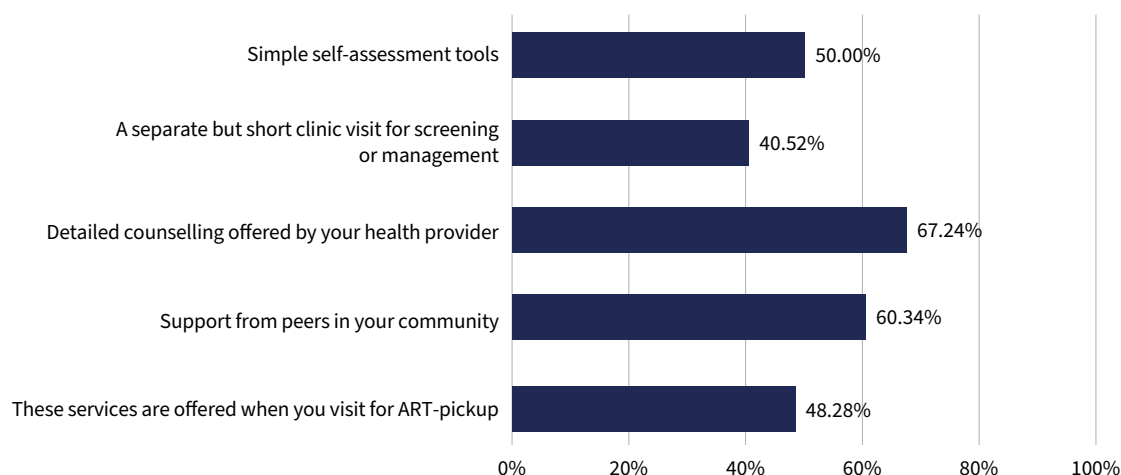
Graph 1. The following question evaluates your relationship with your health care provider: select one response you agree with the most for each statement listed (only one response per row).

Answered: 117 Skipped: 158



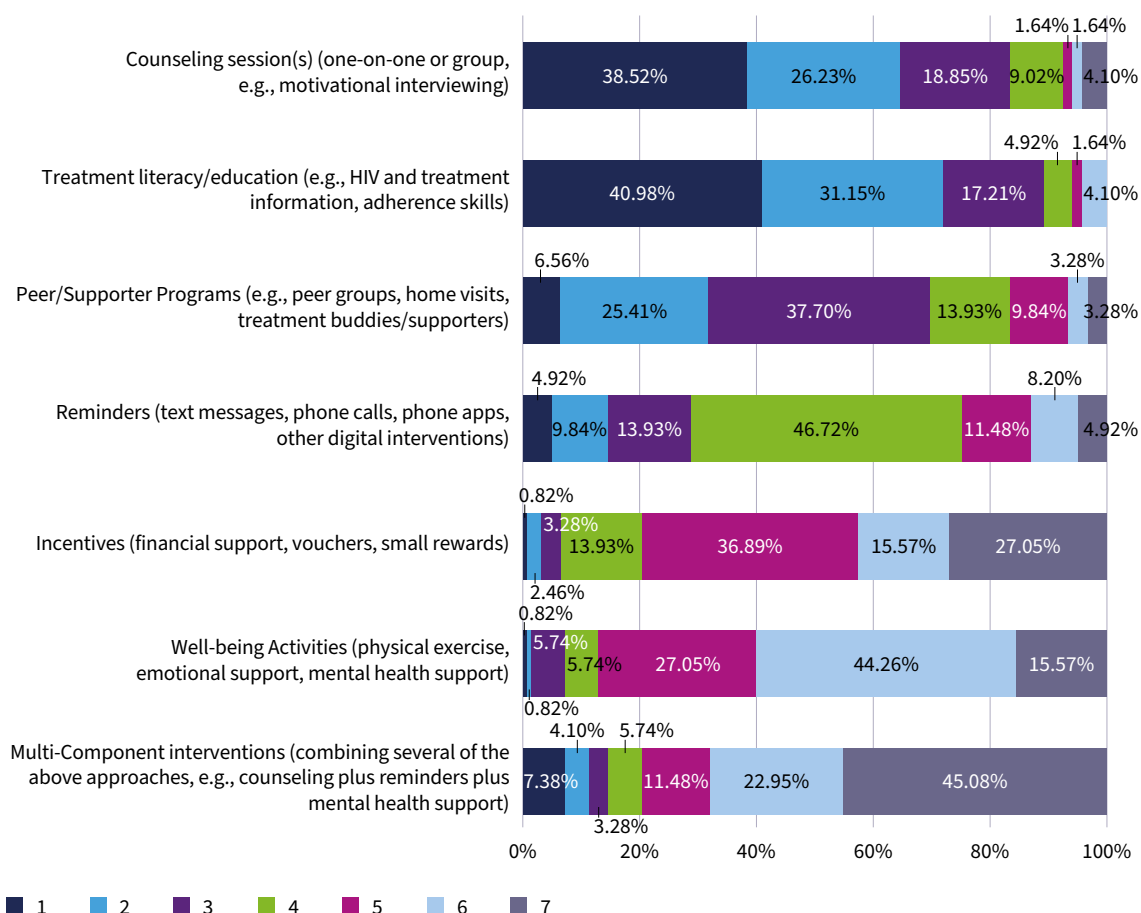
Graph 2. What do you feel will make it easier for you to benefit from NCDs or mental health services? (Mark all that apply).

Answered: 116 Skipped: 159



Graph 3. Which types of adherence support do you consider most helpful for you to take your HIV treatment consistently. (Rank order list, start from most preferable to least preferable).

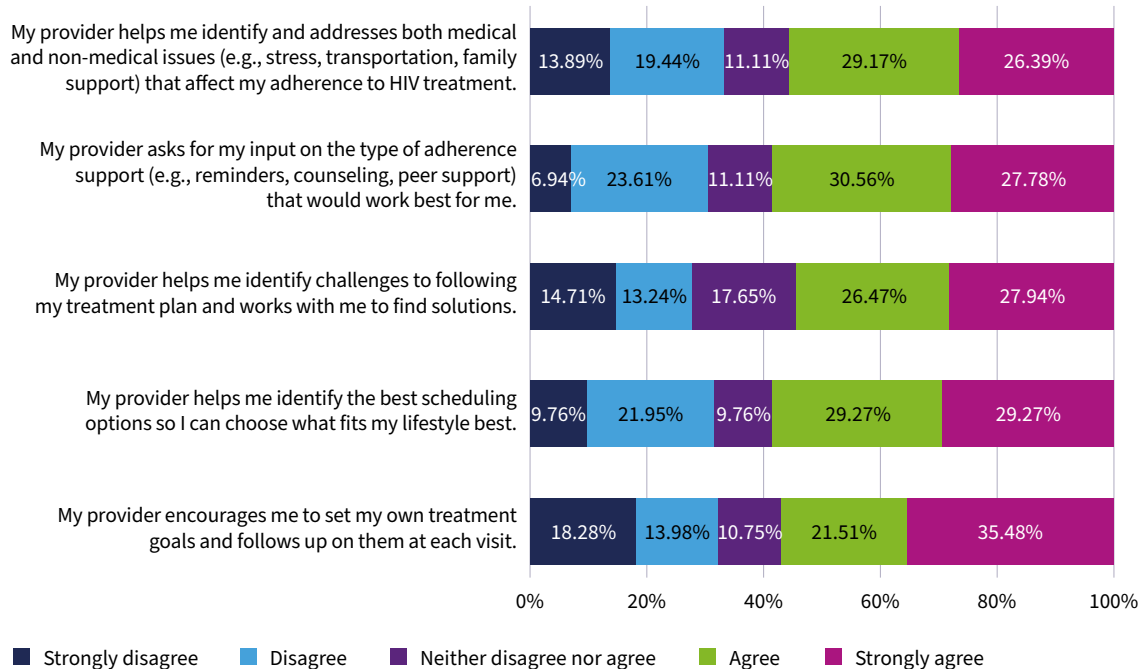
Answered: 122 Skipped: 153



Types of adherence support	Weighted average
Counseling session(s) (one-on-one or group, e.g., motivational interviewing)	5.70
Treatment literacy/ education (e.g., HIV and treatment information, adherence skills)	5.93
Peer/Supporter Programs (e.g., peer groups, home visits, treatment buddies/supporters)	4.82
Reminders (text messages, phone calls, phone apps, other digital interventions)	4.06
Incentives (financial support, vouchers, small rewards)	2.61
Well-being Activities (physical exercise, emotional support, mental health support).	2.48
Multi-Component interventions (combining several of the above approaches).	2.41

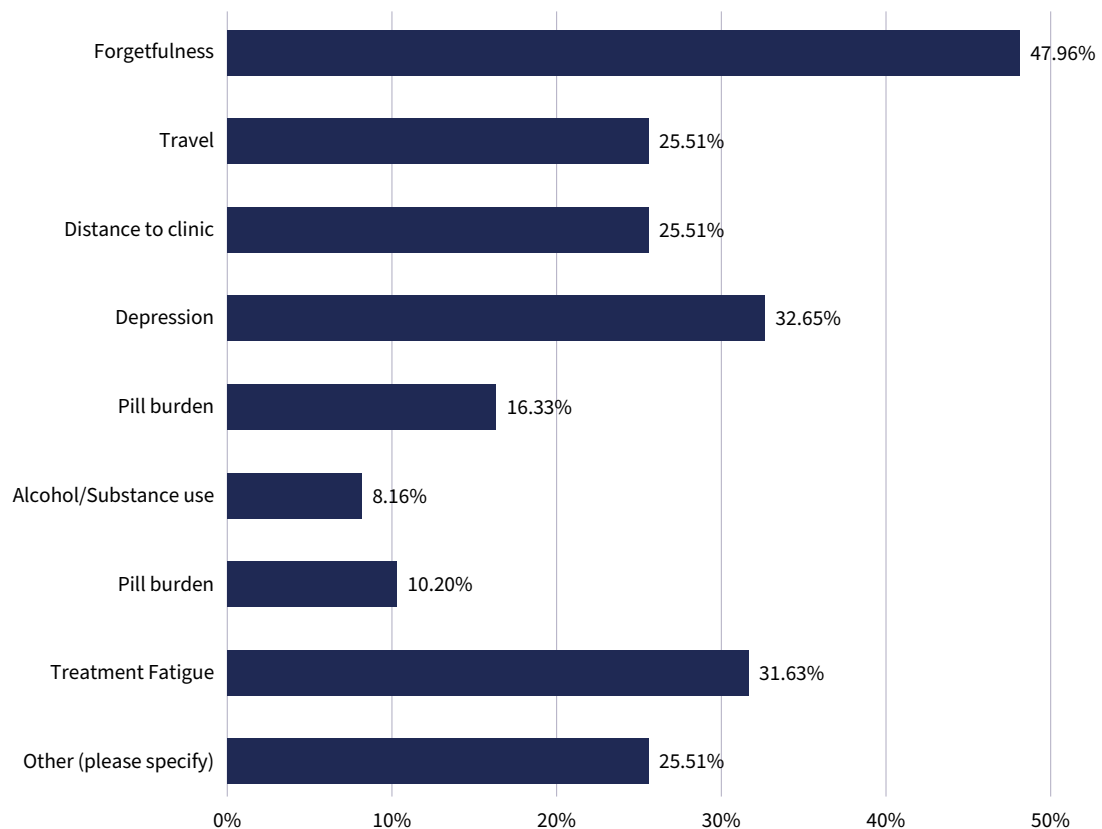
Graph 4. Please read and select how much you agree or disagree with the statements below, informing us how health care providers can better involve you in treatment adherence decisions to enhance your adherence? (One response per row).

Answered: 116 Skipped: 159



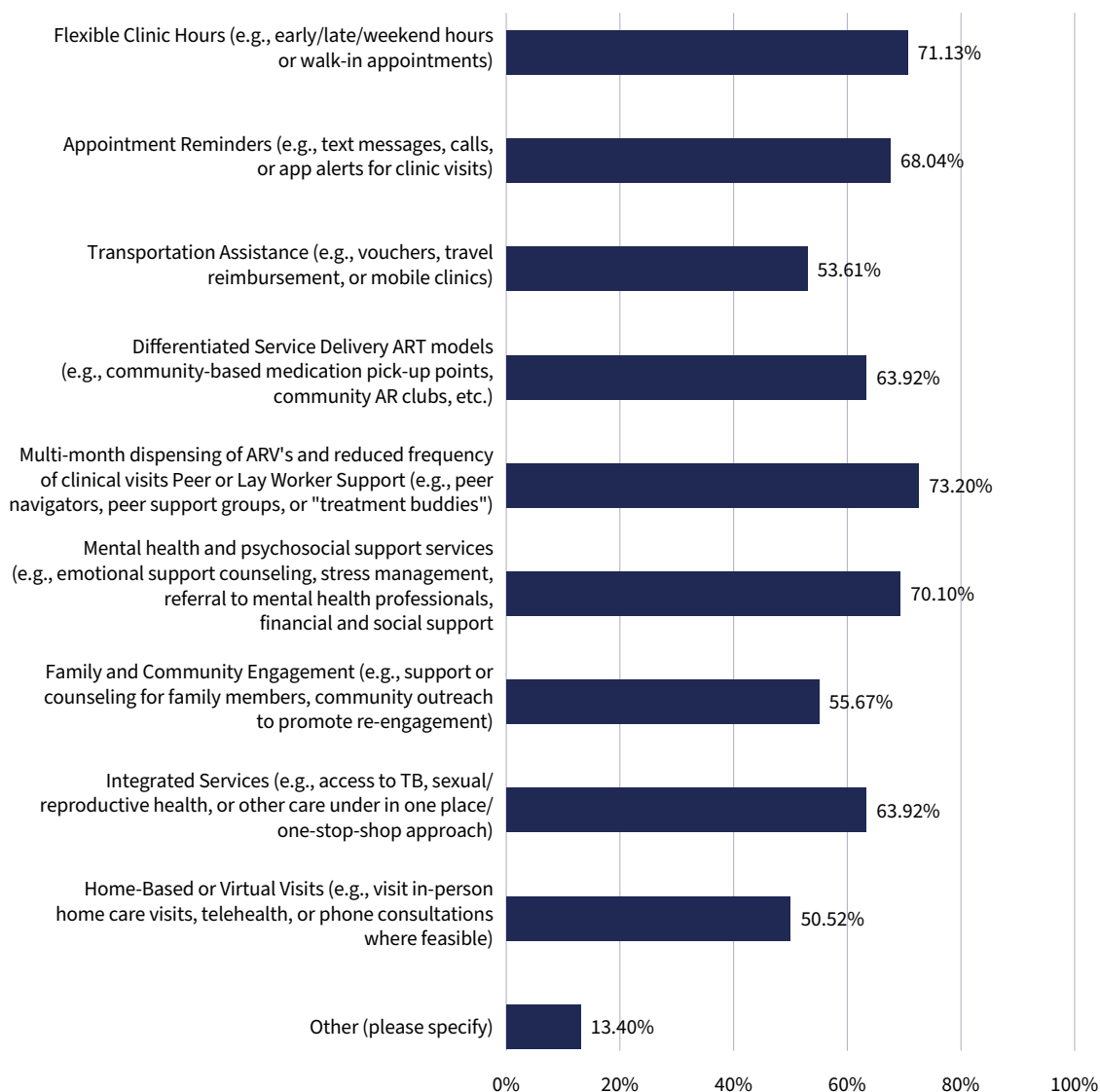
Graph 5. Which challenges do you face in adhering to your HIV treatment? (Select all that apply).

Answered: 98 Skipped: 177



Graph 6. What types of support would you like to receive to be able to re-engage with care and treatment? (Mark all that you think should be provided to you or you feel are important).

Answered: 97 Skipped: 178



**Global HIV, Hepatitis and Sexually
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